

150mm.

LAREYA 20 mg Film-Coated Tablet

COMPOSITION

Each film-coated tablet contains Leflunomide 20 mg.

DESCRIPTION

Light yellow biconvex triangular film-coated tablet debossed with "SPS" on one side and plain on the other.

PHARMACODYNAMICS

Leflunomide is a disease-modifying antirheumatic agent with antiproliferative properties. It is effective in animal models of arthritis and of other autoimmune diseases and transplantation, mainly if administered during the sensitisation phase. It has immunomodulating / immunosuppressive characteristics, acts as an antiproliferative agent, and displays anti-inflammatory properties. Leflunomide exhibits the best protective effects on animal models of autoimmune diseases when administered in the early phase of the disease progression. *In vivo*, it is rapidly and almost completely metabolised to A771726 which is active *in vitro*, and is presumed to be responsible for the therapeutic effect. A771726, the active metabolite of leflunomide, inhibits the human enzyme dihydropyrimidine dehydrogenase (DPYDH) and exhibits antiproliferative activity.

PHARMACOKINETICS

Absorption: Following administration, leflunomide is rapidly and almost completely metabolised by the opening of the isoxazole ring to form the active metabolite, A771726, by first-pass metabolism in the gut wall and liver. The time to peak plasma concentrations of A771726 is very variable; peak plasma levels can occur between 1 hour and 24 hours after single administration. Leflunomide can be administered with food, since the extent of absorption is comparable in the fed and fasting state. Due to the very long half-life of A771726 (approximately 2 weeks), a loading dose of 100 mg for 3 days was used to facilitate the rapid attainment of steady-state levels of A771726. Without a loading dose, it is estimated that attainment of steady-state plasma concentrations would require nearly 2 months of dosing.

Distribution: A771726 is extensively bound to protein (albumin) in human plasma. The extensive protein binding of A771726 could lead to displacement of other highly-bound drugs. There is no indication that this effect is of clinical relevance. Consistent with extensive protein binding A771726 has a low apparent volume of distribution. There is no preferential uptake in erythrocytes.

Biotransformation: Leflunomide is metabolised to one primary (A771726) and many minor metabolites including TFMA (4-trifluoromethylalanine). The metabolic biotransformation of leflunomide to A771726 and subsequent metabolism of A771726 is not controlled by a single enzyme and has been shown to occur in microsomal and cytosolic cellular fractions. At the present time the specific site of leflunomide metabolism is unknown.

Elimination: Elimination of A771726 is slow and characterised by an apparent clearance of about 31 ml/hr. The elimination half-life in patients is approximately 2 weeks. A771726 was still detectable in urine and faeces 36 days after a single administration. The principal urinary metabolites were glucuronide products derived from leflunomide (mainly in 0 to 24 hour samples) and an oxallic acid derivative of A771726. The principal faecal component was A771726.

Administration of an oral suspension of activated powdered charcoal or colestyramine leads to a rapid and significant increase in A771726 elimination rate and decline in plasma concentrations. This is thought to be achieved by a gastrointestinal dialysis mechanism and/or by interrupting enterohepatic recycling. Consequently, where rapid elimination of this metabolite is necessary, any of these medicines may be used to carry out an elimination procedure as described below (see WARNINGS AND PRECAUTIONS).

Pharmacokinetic data in elderly (>65 years) are limited but consistent with pharmacokinetics in younger adults. In patients with renal impairment, pharmacokinetic parameters gave no indication of accumulation. No data are available regarding treatment of patients with hepatic impairment. The active metabolite A771726 is extensively protein bound and cleared via hepatic metabolism and biliary secretion. These processes may be affected by hepatic dysfunction.

INDICATIONS

LAREYA is indicated for the treatment of adult patients with:

- active rheumatoid arthritis as a "disease-modifying antirheumatic drug" (DMARD), to improve physical function, and
- active psoriatic arthritis.

Recent or concurrent treatment with hepatotoxic or haematotoxic DMARDs (e.g. methotrexate) may result in an increased risk of serious adverse reactions; therefore, the initiation of leflunomide treatment has to be carefully considered regarding these benefit/risk aspects. Moreover, switching from leflunomide to another DMARD without following the washout procedure may also increase the risk of serious adverse reactions even for a long time after the switching.

RECOMMENDED DOSAGE

The treatment should be initiated and supervised by specialists experienced in the treatment of rheumatoid arthritis and psoriatic arthritis.

Alanine aminotransferase (ALT) or serum glutamylpyruvate transferase (SGPT) and a complete blood cell count, including a differential white blood cell count and a platelet count, must be checked simultaneously and with the same frequency:

- before initiation of leflunomide,
- every 2 weeks during the first 6 months of treatment, and
- every 8 weeks thereafter.

Posology

- In rheumatoid arthritis: leflunomide therapy is usually started with a loading dose of 100 mg once daily for 3 days. Omission of the loading dose may decrease the risk of adverse events. The recommended maintenance dose is leflunomide 10 mg to 20 mg once daily depending on the severity (activity) of the disease.
- In psoriatic arthritis: leflunomide therapy is started with a loading dose of 100 mg once daily for 3 days. The recommended maintenance dose is leflunomide 20 mg once daily.

The therapeutic effect usually starts after 4 to 6 weeks and may further improve up to 4 to 6 months. There is no dose adjustment recommended in patients with mild renal insufficiency. No dosage adjustment is required in patients above 65 years of age.

Paediatric population

Leflunomide is not recommended for use in patients below 18 years since efficacy and safety in juvenile rheumatoid arthritis (JRA) have not been established.

METHOD OF ADMINISTRATION

LAREYA tablets should be swallowed whole with sufficient amounts of liquid. The extent of leflunomide absorption is not affected if it is taken with food.

CONTRAINDICATIONS

- Hypersensitivity (especially previous Stevens-Johnson syndrome, toxic epidermal necrolysis, erythema multiforme) to the active substance, to the principal active metabolite teriflunomide or to any of the excipients.
- Patients with impairment of liver function.
- Patients with severe immunodeficiency states, e.g. AIDS.
- Patients with significantly impaired bone marrow function or significant anaemia, leucopenia, neutropenia or thrombocytopenia due to causes other than rheumatoid or psoriatic arthritis.
- Patients with serious infections.
- Patients with moderate to severe renal insufficiency, because insufficient clinical experience is available in this patient group.
- Patients with severe hypoproteinaemia, e.g. in nephrotic syndrome.
- Pregnant women, or women of childbearing potential who are not using reliable contraception during treatment with leflunomide and thereafter as long as the plasma levels of the active metabolite are above 0.02 mg/L. Pregnancy must be excluded before start of treatment with leflunomide.
- Breast-feeding women.

WARNINGS AND PRECAUTIONS

Concomitant administration of hepatotoxic or haematotoxic DMARDs (e.g. methotrexate) is not advisable. The active metabolite of leflunomide, A771726, has a long half-life, usually 1 to 4 weeks. Serious undesirable effects might occur (e.g. hepatotoxicity, haematotoxicity or allergic reactions, see below), even if the treatment with leflunomide has been stopped. Therefore, when such toxicities occur or if for any other reason A771726 needs to be cleared rapidly from the body, the washout procedure has to be followed. The procedure may be repeated as clinically necessary. For washout procedures and other recommended actions in case of desired or unintended pregnancy, see PREGNANCY AND LACTATION.

Liver reactions

Rare cases of severe liver injury, including cases with fatal outcome, have been reported during treatment with leflunomide. Most of the cases occurred within the first 6 months of treatment. Co-treatment with other hepatotoxic medicinal products was frequently present. It is considered essential that monitoring recommendations are strictly adhered to.

ALT (SGPT) must be checked before initiation of leflunomide and at the same frequency as the complete blood cell count (every 2 weeks) during the first 6 months of treatment and every 8 weeks thereafter.

For ALT (SGPT) elevations between 2- and 3-fold the upper limit of normal, dose reduction from 20 mg to 10 mg may be considered and monitoring must be performed weekly. If ALT (SGPT) elevations of more than 2-fold the upper limit of normal persist or if ALT elevations of more than 3-fold the upper limit of normal are present, leflunomide must be discontinued and washout procedures initiated. It is recommended that monitoring of liver enzymes be maintained after discontinuation of leflunomide treatment, until liver enzyme levels have normalised.

Due to a potential for additive hepatotoxic effects, it is recommended that alcohol consumption be avoided during treatment with leflunomide.

Since the active metabolite of leflunomide, A771726, is highly protein bound and cleared via hepatic metabolism and biliary secretion, plasma levels of A771726 are expected to be increased in patients with hypoproteinaemia. Leflunomide is contraindicated in patients with severe hypoproteinaemia or impairment of liver function.

Haematological reactions

Together with ALT, a complete blood cell count, including differential white blood cell count and platelets, must be performed before start of leflunomide treatment as well as every 2 weeks for the first 6 months of treatment and every 8 weeks thereafter.

In patients with pre-existing anaemia, leucopenia, and/or thrombocytopenia as well as in patients with impaired bone marrow function or those at risk of bone marrow suppression, the risk of haematological disorders is increased. If such effects occur, a washout (see below) to reduce plasma levels of A771726 should be considered. In case of severe haematological reactions, including pancytopenia, LAREYA and any concomitant myelosuppressive treatment must be discontinued and a leflunomide washout procedure initiated.

Combinations with other treatments

The use of leflunomide with antimalarials used in rheumatic diseases (e.g. chloroquine and hydroxychloroquine), intramuscular or oral gold, D-penicillamine, azathioprine and other immunosuppressive agents including Tumour Necrosis Factor alpha-inhibitors has not been adequately studied up to now in randomised trials (with the exception of methotrexate). The risk associated with combination therapy, in particular in long-term treatment, is unknown. Since such therapy can lead to additive or even synergistic toxicity (e.g. hepato- or haematotoxicity), combination with another DMARD (e.g. methotrexate) is not advisable.

Co-administration of teriflunomide with leflunomide is not recommended, as leflunomide is the parent compound of teriflunomide.

Switching to other treatments

As leflunomide has a long persistence in the body, a switching to another DMARD (e.g. methotrexate) without performing the washout procedure (see below) may raise the possibility of additive risks even for a long time after the switching (i.e. kinetic interaction, organ toxicity).

Similarly, recent treatment with hepatotoxic or haematotoxic medicinal products (e.g. methotrexate) may result in increased side effects; therefore, the initiation of leflunomide treatment has to carefully be considered regarding these benefit/risk aspects and closer monitoring is recommended in the initial phase after switching.

Skin reactions

In case of ulcerative stomatitis, leflunomide administration should be discontinued.

Very rare cases of Stevens-Johnson syndrome or toxic epidermal necrolysis and Drug Reaction with Eosinophilia and Systemic Symptoms (DRESS) have been reported in patients treated with leflunomide. As soon as skin and/or mucosal reactions are observed which raise the suspicion of such severe reactions, LAREYA and any other possibly associated treatment must be discontinued, and a leflunomide washout procedure initiated immediately.

A complete washout is essential in such cases. In such cases re-exposure to leflunomide is contraindicated. Pruritic rash and worsening of psoriasis have been reported after the use of leflunomide. Treatment withdrawal may be considered taking into account patient's disease and past history.

Infections

It is known that medicinal products with immunosuppressive properties - like leflunomide - may cause patients to be more susceptible to infections, including opportunistic infections. Infections may be more severe in nature and may, therefore, require early and vigorous treatment. In the event that severe, uncontrolled infections occur, it may be necessary to interrupt leflunomide treatment and administer a washout procedure as described below.

Immunosuppressed patients are at increased risk for opportunistic infections, including activation of latent viral infections. These include BK virus associated nephropathy which has been observed in patients receiving immunosuppressants. These infections may lead to serious, including fatal outcomes.

Rare cases of Progressive Multifocal Leukoencephalopathy (PML) have been reported in patients receiving leflunomide among other immunosuppressants.

Before starting treatment, all patients should be evaluated for active and inactive ("latent") tuberculosis, as per local recommendations. This can include medical history, possible previous contact with tuberculosis, and/or appropriate screening such as lung x-ray, tuberculin test and/or interferon-gamma release assay, as applicable. Prescribers are reminded of the risk of false negative tuberculin skin test results, especially in patients who are severely ill or immunocompromised. Patients with a history of tuberculosis should be carefully monitored because of the possibility of reactivation of the infection.

Respiratory reactions

Interstitial lung disease, as well as rare cases of pulmonary hypertension have been reported during treatment with leflunomide. The risk of their occurrence can be increased in patients with a history of interstitial lung disease.

Interstitial lung disease is a potentially fatal disorder, which may occur acutely during therapy. Pulmonary symptoms, such as cough and dyspnoea, may be a reason for discontinuation of the therapy and for further investigation, as appropriate.

Peripheral neuropathy

Cases of peripheral neuropathy have been reported in patients receiving leflunomide. Most patients improved after discontinuation of leflunomide. However there was a wide variability in final outcome, i.e. in some patients the neuropathy resolved and some patients had persistent symptoms. Age older than 60 years, concomitant neurotoxic medications, and diabetes may increase the risk for peripheral neuropathy. If a patient taking leflunomide develops a peripheral neuropathy, consider discontinuing leflunomide therapy and performing the drug elimination procedure.

Colitis

Colitis, including microscopic colitis has been reported in patients treated with leflunomide. In patients on leflunomide treatment presenting unexplained chronic diarrhoea appropriate diagnostic procedures should be performed.

Blood pressure

Blood pressure must be checked before the start of leflunomide treatment and periodically thereafter.

Procreation (recommendations for men)

Male patients should be aware of the possible male-mediated foetal toxicity. Reliable contraception during treatment with leflunomide should also be guaranteed. There are no specific data on the risk of male-mediated foetal toxicity. To minimise any possible risk, men wishing to father a child should consider discontinuing use of leflunomide and taking colestyramine 8 g 3 times daily for 11 days or 50 g of activated powdered charcoal 4 times daily for 11 days.

In either case the A771726 plasma concentration is then measured for the first time. Thereafter, the A771726 plasma concentration must be determined again after an interval of at least 14 days. If both plasma concentrations are below 0.02 mg/L, and after a waiting period of at least 3 months, the risk of foetal toxicity is very low.

Interference with determination of ionised calcium levels

The measurement of ionised calcium levels might show falsely decreased values under treatment with leflunomide and/or teriflunomide (the active metabolite of leflunomide) depending on the type of ionised calcium analyser used (e.g. blood gas analyser). Therefore, the plausibility of observed decreased ionised calcium levels needs to be questioned in patients under treatment with leflunomide or teriflunomide. In case of doubtful measurements, it is recommended to determine the total albumin adjusted serum calcium concentration.

Washout procedure

Colestyramine 8 g is administered 3 times daily. Alternatively, 50 g of activated powdered charcoal is administered 4 times daily. Duration of a complete washout is usually 11 days. The duration may be modified depending on clinical or laboratory variables.

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Lactose intolerance

LAREYA contains lactose. Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicinal product.

Effects on ability to drive and use machines

In the case of side effects such as dizziness, the patient's ability to concentrate and to react properly may be impaired. In such cases patients should refrain from driving cars and using machines.

DRUG INTERACTIONS

Interactions studies have only been performed in adults.

Hepatotoxic and haematotoxic drugs

Increased side effects may occur in case of recent or concomitant use of hepatotoxic or haematotoxic medicinal products or when leflunomide treatment is followed by such medicinal products without a washout period. Therefore, closer monitoring of liver enzymes and haematological parameters is recommended in the initial phase after switching.

Methotrexate

Increase in liver enzymes was reported in clinical studies with co-administration of leflunomide and methotrexate. Therefore, liver enzymes should be monitored closely in the initial phase after a changeover to methotrexate has been demonstrated.

Vaccinations

No clinical data are available on the efficacy and safety of vaccinations under leflunomide treatment. Vaccination with live attenuated vaccines is, however, not recommended. The long half-life of leflunomide should be considered when contemplating administration of a live attenuated vaccine after stopping **LAREYA**.

Warfarin and other coumarin anticoagulants

There have been case reports of increased prothrombin time, when leflunomide and warfarin were co-administered. A pharmacodynamics interaction with warfarin was observed with A771726. Therefore, when warfarin or another coumarin anticoagulant is co-administered, close international normalised ratio (INR) follow-up and monitoring is recommended.

NSAIDs / Corticosteroids

If the patient is already receiving nonsteroidal anti-inflammatory drugs (NSAIDs) and/or corticosteroids, these may be continued after starting leflunomide.

Colestyramine or activated charcoal

It is recommended that patients receiving leflunomide are not treated with colestyramine or activated powdered charcoal because this leads to a rapid and significant decrease in plasma A771726 (the active metabolite of leflunomide) concentration. The mechanism is thought to be by interruption of enterohepatic recycling and/or gastrointestinal dialysis of A771726.

CYP450 inhibitors and inducers

In vitro inhibition studies in human liver microsomes suggest that cytochrome P450 (CYP) 1A2, 2C19 and 3A4 are involved in leflunomide metabolism. An *in vivo* interaction study with leflunomide and cimetidine (non-specific weak cytochrome P450 (CYP) inhibitor) has demonstrated a lack of significant impact on A771726 exposure. Following concomitant administration of a single dose of leflunomide to subjects receiving multiple doses of rifampicin (non-specific cytochrome P450 inducer) A771726 peak levels were increased by approximately 40%, whereas the AUC was not significantly changed. The mechanism of this effect is unclear. Because of the potential for A771726 levels to continue to increase with multiple dosing, caution should be exercised if patients are to be receiving both leflunomide and rifampicin.

Oral contraceptives

When leflunomide was administered concomitantly with a low-dose triphasic oral contraceptive to healthy female volunteers, there was no effect on either the anti-ovulatory activity of the contraceptive or the pharmacokinetics of A771726.

The following pharmacokinetic and pharmacodynamic interaction studies were conducted with A771726 (principal active metabolite of leflunomide). As similar drug-drug interactions cannot be excluded for leflunomide at recommended doses, the following study results and recommendations should be considered in patients treated with leflunomide:

• Effect on repaglinide (CYP2C8 substrate)

There was an increase in mean repaglinide C_{max} and AUC following repeated doses of A771726, suggesting that A771726 is an inhibitor of CYP2C8 *in vivo*. Therefore, monitoring patients with concomitant use of medicinal products metabolised by CYP2C8, such as repaglinide, pacitaxel, pioglitazone or rosiglitazone, is recommended as they may have higher exposure.

• Effect on caffeine (CYP1A2 substrate)

Repeated doses of A771726 decreased mean C_{max} and AUC of caffeine (CYP1A2 substrate), suggesting that A771726 may be a weak inducer of CYP1A2 *in vivo*. Therefore, medicinal products metabolised by CYP1A2 (such as duloxetine, alosetron, theophylline and tizanidine) should be used with caution during treatment, as it could lead to the reduction of the efficacy of these products.

• Effect on organic anion transporter 3 (OAT3) substrates

There was an increase in mean cefaclor C_{max} and AUC following repeated doses of A771726, suggesting that A771726 is an inhibitor of OAT3 *in vivo*. Therefore, when co-administered with substrates of OAT3, such as cefaclor, benzylpenicillin, ciprofloxacin, indomethacin, ketoprofen, furosemide, cimetidine, methotrexate, zidovudine, caution is recommended.

• Effect on Breast Cancer Resistance Protein (BCRP) and/or organic anion transporting polypeptide B1 and B3 (OATP1B1/B3) substrates

There was an increase in mean rosuvastatin C_{max} and AUC following repeated doses of A771726. However, there was no apparent impact of this increase in plasma rosuvastatin exposure on the HMG-CoA reductase activity. If used together, the dose of rosuvastatin should not exceed 10 mg once daily. For other substrates of BCRP (e.g. methotrexate, topotecan, sulfasalazine, daunorubicin, doxorubicin) and the OATP family especially HMG-CoA reductase inhibitors (e.g. simvastatin, atorvastatin, pravastatin, methotrexate, nateglinide, repaglinide, rifampicin) concomitant administration should also be undertaken with caution. Patients should be closely monitored for signs and symptoms of excessive exposure to the medicinal products and reduction of the dose of these medicinal products should be considered.

• Effect on oral contraceptive (0.03 mg ethinylestradiol and 0.15 mg levonorgestrel)

There was an increase in mean ethinylestradiol C_{max} and AUC_{0-24} and levonorgestrel C_{max} and AUC_{0-24} following repeated doses of A771726. While this interaction is not expected to adversely impact the efficacy of oral contraceptives, consideration should be given to the type of oral contraceptive treatment.

• Effect on warfarin (CYP2C9 substrate)

Repeated doses of A771726 had no effect on the pharmacokinetics of S-warfarin, indicating that A771726 is not an inhibitor or an inducer of CYP2C9. However, a decrease in peak international normalised ratio (INR) was observed when A771726 was co-administered with warfarin as compared with warfarin alone. Therefore, when warfarin is co-administered, close INR follow-up and monitoring is recommended.

PREGNANCY AND LACTATION

Pregnancy

The active metabolite of leflunomide, A771726 is suspected to cause serious birth defects when administered during pregnancy. Leflunomide is contraindicated in pregnancy. Women of childbearing potential have to use effective contraception during and up to 2 years after treatment (see "Waiting period" below) or up to 11 days after treatment (see abbreviated "Washout period" below).

The patient must be advised that if there is any delay in onset of menses or any other reason to suspect pregnancy, they must notify the physician immediately for pregnancy testing, and if positive, the physician and patient must discuss the risk to the pregnancy. It is possible that rapidly lowering the blood level of the active metabolite, by instituting the drug elimination procedure described below, at the first delay of menses may decrease the risk to the foetus from leflunomide.

For women receiving leflunomide treatment and who wish to become pregnant, one of the following procedures is recommended in order to ascertain that the foetus is not exposed to toxic concentrations of A771726 (target concentration below 0.02 mg/L):

Waiting period

A771726 plasma levels can be expected to be above 0.02 mg/L for a prolonged period. The concentration may be expected to decrease below 0.02 mg/L about 2 years after stopping the treatment with leflunomide. After a 2-year waiting period, the A771726 plasma concentration is measured for the first time. Thereafter, the A771726 plasma concentration must be determined again after an interval of at least 14 days. If both plasma concentrations are below 0.02 mg/L, no teratogenic risk is to be expected.

Washout procedure

After stopping treatment with leflunomide:

- Colestyramine 8 g is administered 3 times daily for a period of 11 days,
- Alternatively, 50 g of activated powdered charcoal is administered 4 times daily for a period of 11 days.

However, also following either of the washout procedures, verification by 2 separate tests at an interval of at least 14 days and a waiting period of one-and-a-half months between the first occurrence of a plasma concentration below 0.02 mg/L and fertilisation is required. Women of childbearing potential should be told that a waiting period of 2 years after treatment discontinuation is required before they may become pregnant. If a waiting period of up to approximately 2 years under reliable contraception is considered unpractical, prophylactic institution of a washout procedure may be advisable. Both colestyramine and activated powdered charcoal may influence the absorption of oestrogens and progestogens such that reliable contraception with oral contraceptives may not be guaranteed during the washout procedure with colestyramine or activated powdered charcoal. Use of alternative contraceptive methods is recommended.

Lactation

Animal studies indicate that leflunomide or its metabolites pass into breast milk. Breast-feeding women must, therefore, not receive leflunomide.

SIDE EFFECTS

Infections and infestations

Rare: Severe infections, including sepsis which may be fatal.

Like other agents with immunosuppressive potential, leflunomide may increase susceptibility to infections, including opportunistic infections. Thus, the overall incidence of infections can increase (in particular of rhinitis, bronchitis and pneumonia).

Neoplasms benign, malignant and unspecified (incl. cysts and polyps)

The risk of malignancy, particularly lymphoproliferative disorders, is increased with use of some immunosuppressive agents.

Blood and lymphatic system disorders

Common: Leucopenia (leucocytes >2 G/L)

Uncommon: Anaemia, mild thrombocytopenia (platelets <100 G/L)

Rare: Pancytopenia (probably by antiproliferative mechanism), leucopenia (leucocytes <2 G/L), eosinophilia

Very rare: Agranulocytosis

Recent, concomitant or consecutive use of potentially myelotoxic agents may be associated with a higher risk of haematological effects.

Immune system disorders

Common: Mild allergic reactions

Very rare: Severe anaphylactic/anaphylactoid reactions, vasculitis, including cutaneous necrotizing vasculitis

Metabolism and nutrition disorders

Common: CPK increased

Uncommon: Hypokalaemia, hyperlipidaemia, hypophosphataemia

Rare: LDH increased

Not known: Hypouricaemia

Psychiatric disorders

Uncommon: Anxiety

Nervous system disorders

Common: Paraesthesia, headache, dizziness, peripheral neuropathy

Cardiac disorders

Common: Mild increase in blood pressure

Rare: Severe increase in blood pressure

Respiratory, thoracic and mediastinal disorders

Rare: Interstitial lung disease (including interstitial pneumonitis), which may be fatal.

Not known: Pulmonary hypertension

Gastrointestinal disorders

Common: Colitis including microscopic colitis such as lymphocytic colitis, collagenous colitis, diarrhoea, nausea, vomiting, oral mucosal disorders (e.g. aphthous stomatitis, mouth ulceration), abdominal pain

Uncommon: Taste disturbances

Very rare: Pancreatitis

Hepatobiliary disorders

Common: Elevation of liver parameters (transaminases [especially ALT], less often gamma-GT, alkaline phosphatase, bilirubin)

Rare: Hepatitis, jaundice/cholestasis

Very rare: Severe liver injury such as hepatic failure and acute hepatic necrosis that may be fatal.

Skin and subcutaneous tissue disorders

Common: Increased hair loss, eczema, rash (including maculopapular rash), pruritus, dry skin

Uncommon: Urticaria

Very rare: Toxic epidermal necrolysis, Stevens-Johnson syndrome, erythema multiforme

Not known: Cutaneous lupus erythematosus, pustular psoriasis or worsening psoriasis, Drug Reaction with Eosinophilia and Systemic Symptoms (DRESS)

Musculoskeletal and connective tissue disorders

Common: Tenosynovitis

Uncommon: Tendon rupture

Renal and urinary disorders

Not known: Renal failure

Reproductive system and breast disorders

Not known: Marginal (reversible) decreases in sperm concentration, total sperm count and rapid progressive motility

General disorders and administration site conditions

Common: Anorexia, weight loss (usually insignificant), asthenia

SYMPTOMS AND TREATMENT OF OVERDOSAGE

Symptoms: There have been reports of chronic overdose in patients taking leflunomide at daily doses up to 5 times the recommended daily dose, and reports of acute overdose in adults and children. There were no adverse events reported in the majority of case reports of overdose. Adverse events consistent with the safety profile for leflunomide were: abdominal pain, nausea, diarrhoea, elevated liver enzymes, anaemia, leucopenia, pruritus and rash.

Management: In the event of an overdose or toxicity, colestyramine or charcoal is recommended to accelerate elimination. Colestyramine given orally at a dose of 8 g 3 times a day for 24 hours to three healthy volunteers decreased plasma levels of A771726 by approximately 40% in 24 hours and by 49% to 65% in 48 hours. Administration of activated charcoal (powder made into a suspension) orally or via nasogastric tube (50 g every 6 hours for 24 hours) has been shown to reduce plasma concentrations of the active metabolite A771726 by 37% in 24 hours and by 48% in 48 hours. These washout procedures may be repeated if clinically necessary. Studies with both haemodialysis and CAPD (chronic ambulatory peritoneal dialysis) indicate that A771726, the primary metabolite of leflunomide, is not dialysable.

PACK SIZE

Aluminium blister packs of 30 film-coated tablets.

STORAGE CONDITION

Store below 30°C and protect from light.

Keep out of reach of children.

Jauhi daripada kanak-kanak.

SHELF-LIFE: Please refer to the outer box label.

MANUFACTURED BY:
Siprasit Pharma Co., Ltd.
216 Moo 6, Suanluang, Krathum Baen,
Samut Sakhon 74110, Thailand.

Date of Revision: August 2022

PIL0012 Rev:0

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